BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. No patient dates are needed; agree who arranges and reviews checks.

Check	Before starting	Induction Day 1	Induction Days 2–21	Maintenance review	After treatment
Bloods and hormones	full blood count (FBC), liver/kidney, glucose, thyroid, hepatitis B/C and GFR baseline	Review results and current symptoms	FBC, liver/kidney and glucose each cycle; thyroid every 3–6 weeks	Continue cycle bloods and thyroid checks	Follow unresolved problems and the team's plan
Treatment and routes	Agree intravenous (IV)/ subcutaneous (SC) atezolizumab and IV versus IV/oral etoposide	All three medicines in the source sequence	Etoposide Days 2–3; next cycle usually after 21 days	Carboplatin and etoposide stop after up to four cycles	Agree follow-up and next treatment decisions
Support medicines	Prepare one reconciled day-by-day anti-sickness plan	Carboplatin-specific high-risk cover	Account for etoposide Days 2–3 and two days afterwards	No automatic continuation of chemotherapy prophylaxis	Continue only individually prescribed medicines
Immune effects and cancer review	Baseline breathing, bowel, skin and endocrine symptoms; alert card	Report administration reactions immediately	Report new cough, diarrhoea, rash or unusual weakness promptly	Review benefit and symptoms; scans per consultant	Delayed immune effects remain possible

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00689 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Atezolizumab + carboplatin + etoposide: treatment and checks

First-line extensive-stage small-cell lung cancer

Induction: atezolizumab and carboplatin on Day 1, with etoposide on Days 1–3, repeated every 21 days for up to four cycles. Maintenance: carboplatin and etoposide stop; atezolizumab alone continues every 21 days while appropriate. The fourth cycle ends about 12 weeks after starting if there are no delays; maintenance has no fixed total duration in this regimen.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Induction Day 1	Induction Days 2–21	Maintenance review	After treatment
Bloods and hormones NCCP 00689	FBC, liver/kidney, glucose, thyroid, hepatitis B/C and GFR baseline	Review results and current symptoms	FBC, liver/kidney and glucose each cycle; thyroid every 3–6 weeks	Continue cycle bloods and thyroid checks	Follow unresolved problems and the team's plan
Treatment and routes NCCP 00689	Agree IV/SC atezolizumab and IV versus IV/oral etoposide	All three medicines in the source sequence	Etoposide Days 2–3; next cycle usually after 21 days	Carboplatin and etoposide stop after up to four cycles	Agree follow-up and next treatment decisions
Support medicines NCCP 00689	Prepare one reconciled day-by-day anti-sickness plan	Carboplatin-specific high-risk cover	Account for etoposide Days 2–3 and two days afterwards	No automatic continuation of chemotherapy prophylaxis	Continue only individually prescribed medicines
Immune effects and cancer review NCCP 00689	Baseline breathing, bowel, skin and endocrine symptoms; alert card	Report administration reactions immediately	Report new cough, diarrhoea, rash or unusual weakness promptly	Review benefit and symptoms; scans per consultant	Delayed immune effects remain possible

What each check is for

1 Before treatment

Before first dose

Confirm suitability, phase and routes

Ask / **act:** Review results, contraindications, medicines and explicit route prescriptions.

Sources: NCCP 00689

2 Phase change

After up to four induction cycles

Chemotherapy ends; immune treatment may continue

Ask / **act:** Confirm atezolizumab schedule and revise anti-sickness medicines without stopping required hormone replacement.

Sources: NCCP 00689

3 After treatment

Completion or change of plan

Delayed effects and follow-up

Ask / **act:** Give follow-up arrangements, alert-card advice and urgent contact details.

Sources: NCCP 00689

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.26